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Lions Outback Vision: Saving Sight With a Lens to the Future

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Author: Luís Gil, Nuno Gil, Leandro S. Pongeluppe

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Abstract

“Carving shells is a tradition and I’m keeping that going. I need my vision to continue doing art for many more years.”

—Bruce Wiggan, a senior Bardi man, Cygnet

This case focuses on intersectoral collaboration across private, public, and nonprofit actors to reduce inequality in eye care in the Pilbara region in Western Australia. As part of a commitment to reduce disparities in eye care, the protagonist, Angus Turner, Director of Lions Outback Vision (LOV), has just delivered the launch speech for a new Sprinter van—a four-wheel-drive van with which LOV is proposing to bring eye care to remote Aboriginal communities. The Aboriginal and Torres Strait Islander groups, Australia’s First Peoples, who have been living on the continent for millennia, are 14 times more likely to suffer diabetes-related blindness and five times more likely to have refractive error blindness than Australia’s White population. Elders from the local Jigalong Aboriginal community, staff of the local Puntukurnu Aboriginal Medical Service, and members of the LOV team all attended the speech in Newman, in the heart of the Pilbara region, and saw the next innovation in remote eye care proposed. As Angus flies back to Broome, also in Western Australia, where the LOV operations are primarily based, his task will be to determine the best approach to convince a diverse group of stakeholders, notably the Government of Western Australia, representatives of the Aboriginal population, and some of the world’s largest mining companies, that his idea can succeed in tackling eye-care

inequality, and in so doing, win AUD 5 million through the Pilbara Challenge prize to further the LOV mission.

Case

Learning Outcomes

By the end of this case study, students should be able to:

- understand alternative intersectoral collaborative approaches to tackling inequity in healthcare at the local level while acknowledging its global scale;
- discuss how stakeholder management and organizational choice to tackle a grand challenge must be sensitive to the institutional environment, technology, and availability of resources in the context;
- understand and appreciate how advancements in AI and Internet coverage create opportunities to find new approaches to resolve the universal problems of division of labor and integration of effort in the context of grand challenges; and
- understand the challenges of aligning shared and individual goals and the arrangements to govern differing stakeholder interests in the pursuit of organizational solutions to tackle grand challenges.

The Pilbara Challenge

Announced in December 2022, the Pilbara Challenge was an initiative by the Western Australian Government ([Figure 1](#)), supported by The Future Health Research and Innovation (FHRI) fund and some of the world's largest mining corporations, including BHP and Rio Tinto. The challenge sought social and healthcare innovations to confront the inequalities suffered in the Pilbara region, a remote area of Western Australia—"Health Care starts with an idea. But it does not stop there. We challenge you to reimagine what it means to deliver regional and remote Health Care," said Stephen Dawson, Minister for Medical Research and Innovation, when announcing the challenge, which came with a prize of AUD 5 million (USD 3.25 million) for the most outstanding solution, to be awarded in October 2024 (Turner, 2024). Ten finalists were selected, and each received a stipend of AUD 200,000 (USD 130,000) to launch and validate a proof-of concept. Lions Outback Vision was one of these finalists with their eyes on the prize.

Figure 1. Australia, Western Australia, and Pilbara Region



Note: Armstrong et al. (2015). www.researchgate.net/figure/Colour-online-A-map-of-the-state-of-Western-Australia-with-major-regions-highlighted_fig1_282633506

Pilbara stands as one region of the vast state of Western Australia, which, if it were a country, would be the 10th largest in the world by size. A sparsely populated and immensely mineral-rich area covering 507,896 km (Fu et al., 2019), the Pilbara region accounts for 20% of the state's size, featuring a delicate balance of purpose-built mining towns on the lands and waters of Australia's First Peoples, ancient landscapes, and Aboriginal communities, each with their own culture, language, beliefs, and practices. Innovative outreach strategies to overcome the geographical barrier and foster cultural connection will be crucial for innovation in healthcare to succeed. Speaking on behalf of Lions Outback Vision (LOV) in their finalist video, Angus talks about how "diabetes is a major cause of blindness, and 98% of it is treatable, and yet only 20% of people living in the Pilbara have had the required eye check." Through the initiative of the four-wheel-drive Sprinter van, Angus and LOV aim for this to be the vehicle that takes the next step in improving healthcare coverage of Aboriginal Australians, harnessing artificial intelligence (AI) and innovations in Internet coverage to achieve the greatest

chance of success through telehealth.

Changing Paradigms of Regional and Remote Healthcare

One of the obstacles to achieving equitable healthcare for First Nations groups in Australia has been the thinly spread-out population. Over one-third live outside of the country's major cities, with 20% of the non-metropolitan population living across 1,500 remote towns or communities, each with less than 5,000 people (Aboriginal Health Council of Western Australia, 2016). This dispersion represents a significant hurdle in supporting these communities with traditional healthcare models, since patients often must travel considerable distances for healthcare services. The psychological hardships of extended time away from family and employment, together with the financial and emotional difficulties that traveling brings, contribute toward high rates of missed healthcare appointments. Further, the traditional model of mobile patients and stable healthcare locations has more significant impacts on Aboriginal patients, who are unable to find adequate transport services. This was exemplified in the 2018–2019 National Aboriginal and Torres Strait Islander Health Survey, in which 243,663 (30%) of Aboriginal Australians aged 15 and over reported they needed to go to a health provider in the last 12 months but did not go, many of them due to the logistical difficulties (Fu et al., 2019). Consequently, the design of healthcare services in these regions must be adapted to uniquely serve the widely differing health needs of regional and remote communities.

Lions Outback Vision

The Pilbara Challenge is not LOV's first rodeo. Founded in 2010 with the backing of the Lions Eye Institute—one of Australia's largest nonprofit eye clinics and vision research centers, set up in 1983—and the University of Western Australia, LOV, under the leadership of Angus Turner, has long been committed to tackling the unique challenges posed by delivering specialist outreach services to vast and remote regions. Since its inception in 2010, LOV has embarked on a remarkable journey, marked by its ground-breaking initiatives. One such trailblazing effort, introduced in 2016, is the Lions Outback "Vision Van," a specialist mobile eye health clinic that circuits Western Australia throughout the year (Figure 2). This distinctive clinic stops in remote towns, often purpose-built by mining companies, for brief intervals of 25 days, extending its outreach to thousands of Aboriginal Australians across 20 remote communities.

Figure 2. Pictures from the Field

Panel A: Lions Outback Vision Sprinter Van Fitted With Starlink Satellite



Panel B: Biometry for Aboriginal Patient



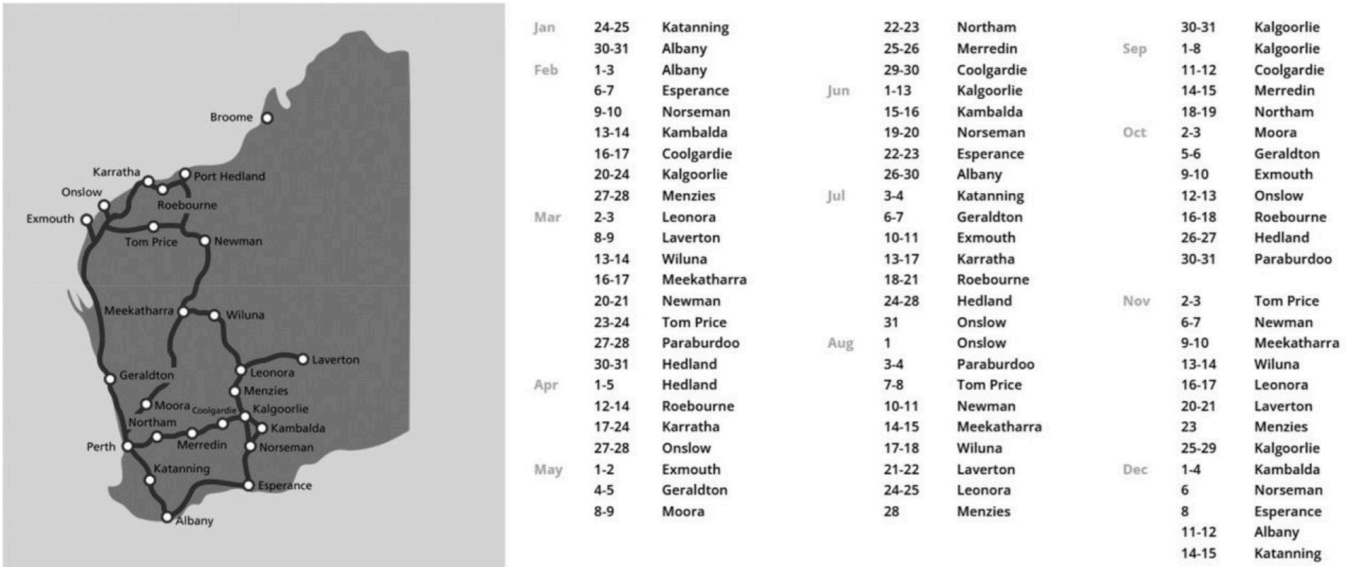
Panel C: The Lions Outback Vision Van**Source:** Authors

LOV's efforts have transformed lives like that of Bruce Wiggan, a senior Bardi man from Cygnet Bay. "It's good for me that I came to get my eye fixed," said Bruce. Behind LOV's success stands Angus Turner, an accomplished Western Australian ophthalmologist. Angus's drive to ensure lasting impact and continuous innovation in remote healthcare is evident from a string of achievements, including a Rhodes scholarship, earning the title of 40 under 40 "First amongst equals" in 2015, and Western Australian of the Year in 2019 (Lions Eye Institute, 2024). Internationally recognized for his contributions as an ophthalmologist and pioneering efforts in teleophthalmology, Angus ensures that LOV harnesses strong partnerships with Aboriginal communities, corporate bodies, and the government to make the vision van successful.

The vision van serves as LOV's "pièce de résistance" and has augmented the existing outreach services to remote Western Australia, traveling up to 25,000 km per year (Fu et al., 2019). The Lions Outback Vision Van is the first mobile clinic in Western Australia to showcase the protocols and principles developed by the Aboriginal Health Council of Western Australia for outback service delivery in Indigenous communities. The team riding in the van consists of a driver who monitors and repairs equipment, an Aboriginal Liaison Officer

(Aboriginal community representatives within the organization), an ophthalmology fellow, and a resident. As a specialist mobile eye health clinic, the van commits to visiting 19 remote communities three times each year, providing comprehensive ophthalmology care for common and uncommon eye diseases, such as cataracts. In addition, LOV runs surgical outreach clinics for eye complications that require procedures to be done in a hospital. Local Aboriginal Medical Services, optometrists, and general practitioners can refer patients suffering from vision problems to the van ahead of its arrival in their community. Plus, the vision van takes walk-ins for people who see the van parked around town. Through its seven years of operation, traveling three circuits of Western Australia a year, the van has amassed a sizable patient list and arranges follow-ups with patients according to its schedule (Figure 3). Many towns where the van stops, such as Port Hedland, Paraburdoo, and Tom Price in the Pilbara, are purpose-built mining towns with Aboriginal communities typically a short distance away. This leads to a duality in the demographic of patients seen—often a half-and-half split of Aboriginal people and industry workers. Prior to the mobile clinic, outreach was limited to diabetic eye screening and telehealth consultations at ophthalmology and optometry clinics, which left many remote communities without access to eye care.

Figure 3. Vision Van Schedule 2023

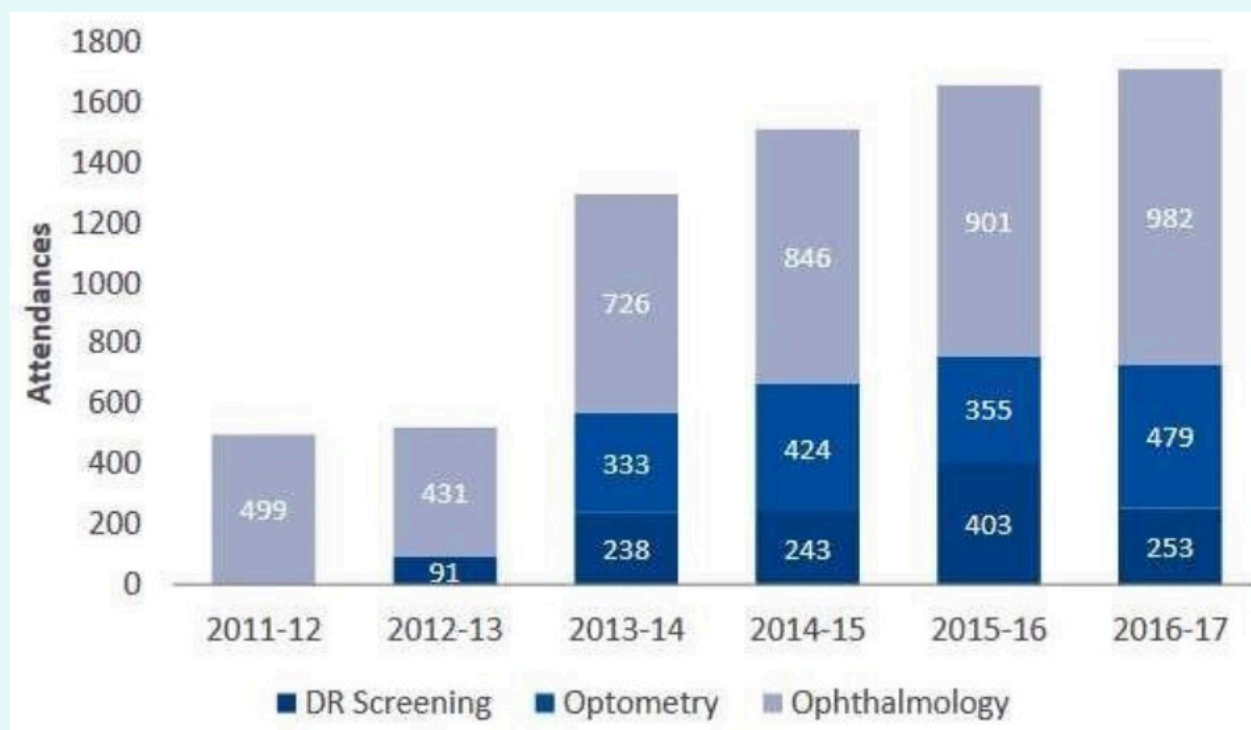


Note: LOV website (<https://www.outbackvision.com.au/>).

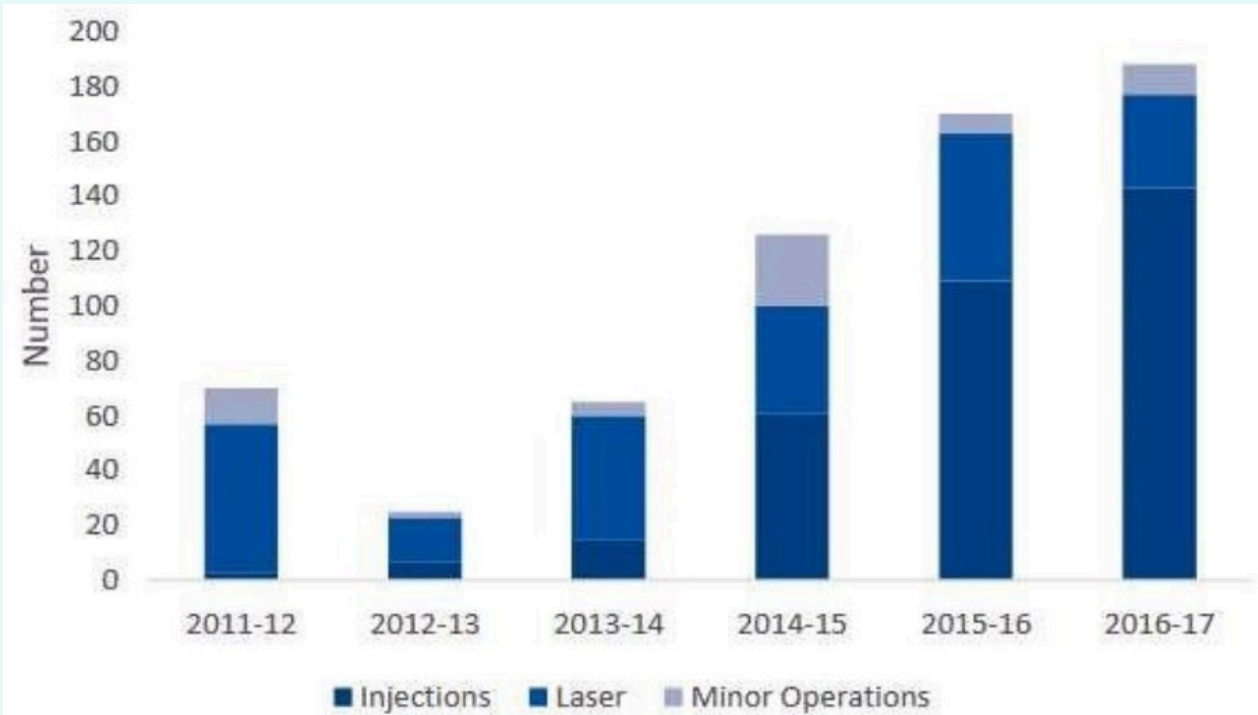
Consisting of three consultation rooms and 14 pieces of specialist equipment, seven of which were previously

unavailable in any of the remote locations, the vision van has effectively reduced the need for patient travel. In cases where patients must be referred urgently for an operation, the state-funded Patient Assisted Transport Scheme (PATS) can provide a subsidy toward the cost of travel and accommodation in urban locations. In just its first year of operating, back in 2016, the van was able to reduce the cost of ophthalmological PATS claims to AUD 1.6 million, compared to the predicted interval of AUD 1.8–2.3 million from the trend over the previous three years (Fu et al., 2019). The impact of the van was further highlighted in the remote town of Derby in the Kimberley region. Patient attendance among a sample population of 199 referrals increased from 50.7% in 2015 to 67.9% (Fu et al., 2019). These figures demonstrate a significant increase in uptake compared to previous data collected across 17 Aboriginal Medical Service sites, averaging 49% (Figure 4). Less clear is the impact of the vision van on the cost structure of patient care, and how that impact differs between Aboriginal vs. non-Aboriginal patients. Still, preliminary figures suggest the costs of ophthalmology services per patient offered through the van are substantially lower than through outreach clinics, which involve coordinated visits by ophthalmologists to regional hospitals or medical centers (Figure 5). The potential economy becomes even more significant considering the logistical challenges of Western Australian regions and their associated costs for treatment (Figure 6).

Figure 4. Evaluating the Impact of the Lions Outback Vision Program

Panel A: LOV Program Attendances by Service Type

Panel B: LOV Program Ophthalmology Procedures and Treatments



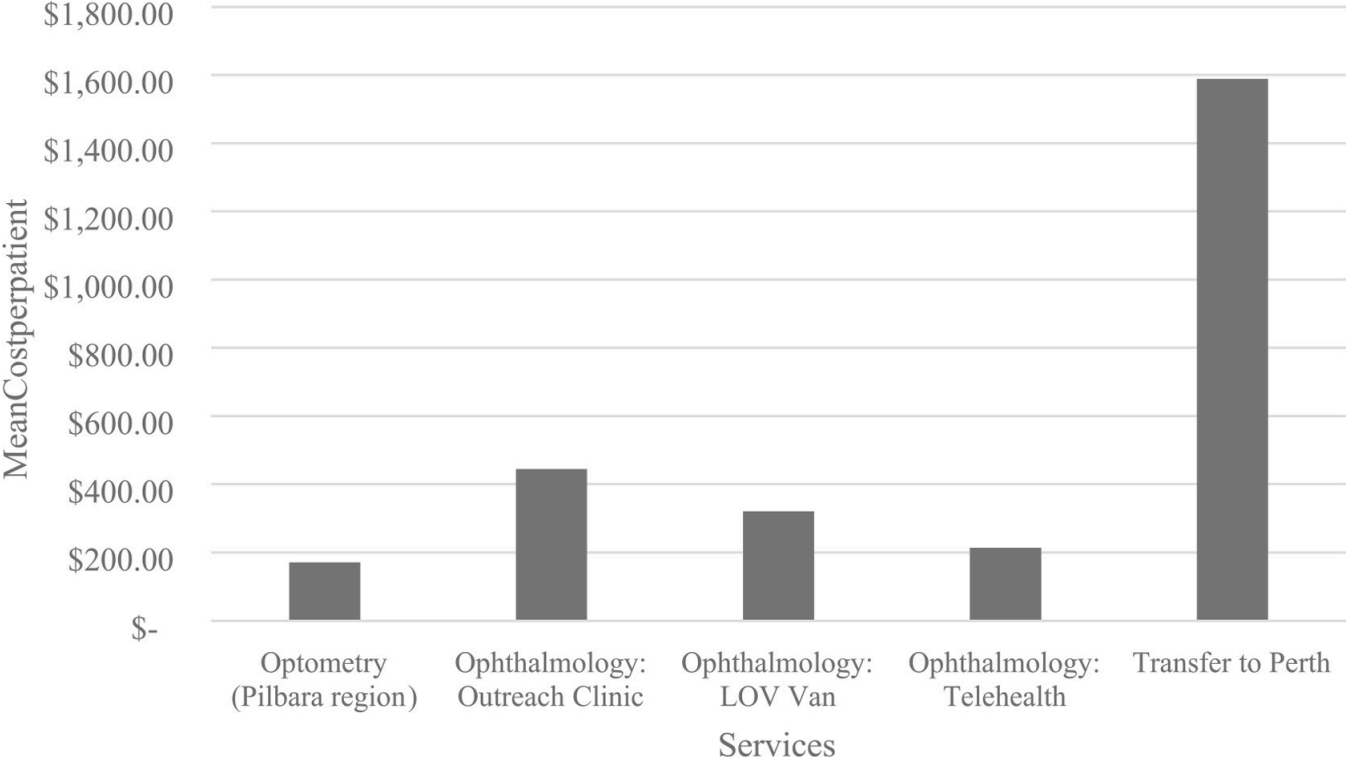
Source: LOV reports. https://www.outbackvision.com.au/wp-content/uploads/2024/03/1712020_line-of-sight-report.pdf

Figure 5. Cost Estimates for Patient Transfers per Region in 2012

Region	Population	Patients transferred	Number of trips	Trips per thousand people	Mean cost per trip (AUD)	Total cost (AUD)
Midwest	52,947	319	567	10.7	257.06	145,755
Great Southern	55,350	641	1,155	20.9	152.62	176,279
Kimberly	34,786	271	410	11.8	810.99	332,509
Pilbara	59,896	298	585	9.8	704.95	412,396
Gascoyne	9,282	54	99	10.7	258.13	25,555
Southwest	72,883	822	1,521	20.9	152.61	232,118
Goldfields	57,403	317	586	10.2	480.95	281,834
Total & on-costs (10%)	342,547	2,722	4,923	15.3	358.94	1,767,091

Source: Razavi et al. (2016, p. 5).

Figure 6. Mean Cost per Patient of the LOV Program in 2020



Source: LOV reports. https://www.outbackvision.com.au/wp-content/uploads/2024/03/1712020_line-of-sight-report.pdf

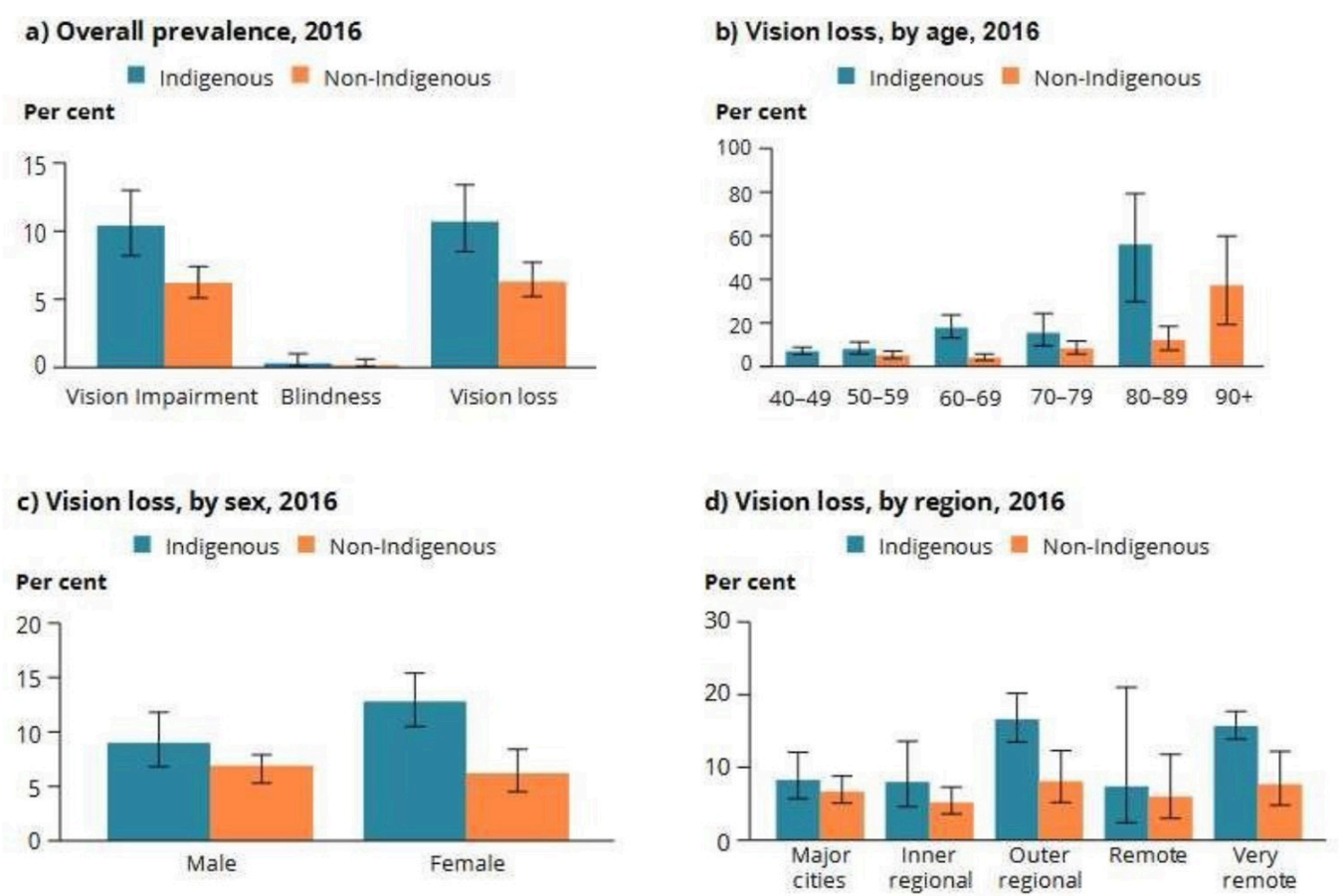
In addition to the vision van, in 2022, LOV established a state-of-the-art hub in Broome to pursue a two-pronged approach toward the lack of eye-care facilities servicing Pilbara and neighboring Kimberley regions. In Australia, nationwide diabetic eye disease screening attempts have been low, with coverage as low as 77.2% in non-Aboriginal and 52.7% in Aboriginal populations (Fu et al., 2019). This is partly due to non-eye-care health professionals lacking the time and confidence to identify the signs of diabetes in the eyes. With PATS and the vision van being the only two services addressing the inequities in eye care, the Broome hub has significantly impacted eye-care coverage in local regions, reducing the need to travel to major urban centers. Built on the land of the Yawuru people, Broome is a coastal town known for its pearling industry and tourism, with a small permanent population of around 14,660 people. It sits 2,200 km away from the nearest major city, Perth, which previously offered the only tertiary eye care in the state. Aboriginal people make up about 23.4% of the Broome population, and there are two Aboriginal Medical Services serving the primary care needs of the local community (Aboriginal Health Council of Western Australia, 2016). The Broome clinic acts as an interdisciplinary base from which the LOV team can deploy its ophthalmology and optometry outreach services to the Kimberley and Pilbara regions and is furnished with specialist equipment rivaling urban tertiary healthcare centers like the one in Perth. The establishment of the Broome hub marked a significant stride in transcending geographical obstacles for remote eye healthcare access. A permanent team working at the hub consists of Angus himself, two ophthalmologists, two Aboriginal Liaison Officers, a permanent optometrist service, and administrative staff. By positioning top-quality services regionally, the Broome hub has enhanced proximity to communities while fostering collaboration with local Aboriginal Medical Services, extending reach to marginalized communities.

Expanding Reach: The Sprinter Van Initiative

The vision van and the Broome hub have represented a cornerstone in bridging the gap in eye care in remote Western Australia. However, there is still a long way to go before expanding outreach. Eye disease and vision problems represent the most reported long-term health problem by Aboriginal Australians, with one-third still reporting eye or sight problems in 2021 (Australian Institute of Health and Welfare, 2021) ([Figure 7](#)). One of the more cost-effective and accessible strategies for addressing inequalities is reducing non-attendance

rates. This issue remains a significant challenge, with Aboriginal Australians being three times more likely to miss medical appointments compared to their non-Aboriginal counterparts. Research conducted by LOV into the primary reasons for these absences identified that 39% of missed appointments were attributed to other commitments, such as being out of town or work obligations. A further 19% cited a lack of transportation as a barrier, while 13% acknowledged forgetfulness as a contributing factor (Copeland et al., 2017). Recognizing that building a culture of attendance is difficult for a visiting outreach service, attendance largely depends on staff at local Aboriginal Medical Services who coordinate patient transport out of the community to the nearby towns where the vision van is parked. The role of the Aboriginal Medical Service staff in identifying reluctant patients, motivating them to attend, providing transport services, and providing patient reminders is thus crucial in increasing coverage of the remote population.

Figure 7. Prevalence of Vision Impairment and Blindness (Vision Loss) by Various Characteristics



Notes:

1. Data have been survey weighted to account for sampling protocol.
2. Error bars show 95% C.I.

Source: NEHS Data (2016); Taylor et al. (2010); Foreman et al. (2017). Report available at: www.aihw.gov.au/getmedia/093acbec-e584-4457-a615-404fb3890379/aihw-ihw261.pdf?v=20230605181820&inline=true

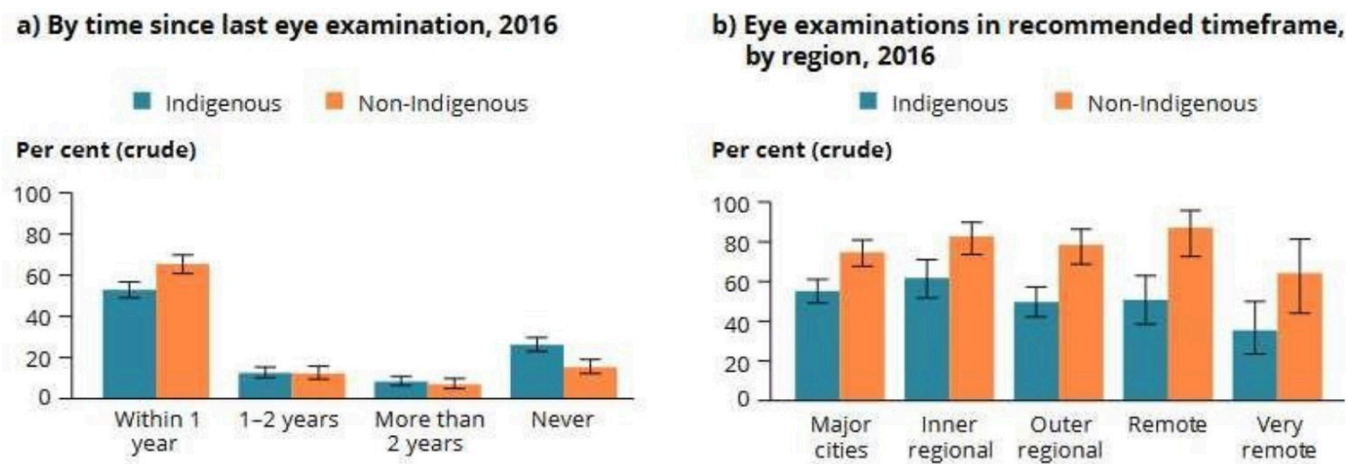
https://www.vision2020australia.org.au/wp-content/uploads/2019/06/National-Eye-Health-Survey_Summary-Report_FL-NAL.pdf

<https://pubmed.ncbi.nlm.nih.gov/20230347/>

<https://pubmed.ncbi.nlm.nih.gov/28689897/>

The vision van is unprecedented because it is an entirely specialist service that provides diabetic eye disease screening as a primary care service. However, a core limitation is its bulky size. As a result, the vision van cannot access Aboriginal communities linked only by unpaved roads, so the coverage of patients screened is affected. Hence, the vision van model relies on local public agencies and nonprofits to transport patients out of the community to the van. Local Aboriginal Medical Services and nonprofits like East Pilbara Support Independence are therefore vital for establishing a culture of attendance, motivating patients, and identifying early on those unlikely to make their appointments to maximize coverage to local communities. A qualitative study run by LOV on non-attendance rates demonstrated the need for transport provision in two ways: first, that nearly one-half of patients who attended cited a reminder from the Aboriginal Medical Services, and most of these reminders came within a week of the appointment. Second, two-thirds of patients relied on the provision of transport to make their appointments, with half of this group indicating they would have missed the appointment if transport was not available (Copeland et al., 2017). Hence, coverage for Aboriginal Australians suffering from diabetes represents an area for significant improvement ([Figure 8](#)).

Figure 8. Proportion of Those With Diabetes Who Had an Eye Examination in The Recommended Time Frame, by Various Characteristics



Notes:

- 1. All figures show crude unadjusted sample proportions.
- 2. Error bars show 95% C.I.

Source: AIHW analysis of the NEHS data 2016. Report available at: <https://www.outbackvision.com.au/wp-content/uploads/2024/03/170530-indigenous-eye-health-measures-2016-2.pdf> (Figure 3.2.5)

Launching the Sprinter van is LOV's initiative to overcome this challenge, transcending the hurdle of limiting factors to bring outreach to local communities. The smaller design of the four-wheel-drive van will enable it to travel down beaten roads, eliminating the need for Aboriginal patients to travel out of their community for eye checks. In addition, the Sprinter van aims to synthesize and trial the latest innovative ophthalmology technology. The van is furnished with an AI-powered OCT color Fundus camera, a technology discussed later, and Starlink connection (SpaceX venture to provide global Internet coverage) so that the van has the capacity for specialist consultations via teleophthalmology. With no other specialist equipment for examination, the van aims to become a mobile primary care screening clinic, increasing the coverage of screening for diabetic Aboriginal patient communities without the need for an ophthalmologist to be physically present.

Virtual Specialist: Enhancing Remote Healthcare with AI and Telemedicine

The future need for AI in ophthalmology is predictable, and AI integration is already underway. There are currently 430 million people worldwide diagnosed as having diabetes, 146 million of whom suffer from diabetic eye disease (Teo et al., 2021). The projected population shows these numbers are predicted to double by 2040 (Teo et al., 2021). AI presents an exciting solution in many fields of medicine to help tackle increasing demand and reduce the burden on clinicians, particularly in ophthalmology, where it has already been trialed in detecting signs of diabetic eye disease from the color fundus (i.e., the back of the eye) images quickly and accurately. These scans inform clinicians on the best therapeutic intervention, if any, and the scheduling of appropriate follow-up at subsequent outreach clinics. Notably, a medically trained specialist is not necessary to operate the machine. Through a partnership with Google on its Automated Retinal Disease Assessment Software, LOV has validated an AI algorithm for Aboriginal Australians, which was also tested in populations in India and Thailand to reduce racial bias (Chia et al., 2024). Another deep-learning system tested by LOV compared the specificity and sensitivity of AI to a retinal specialist, with outcomes showing that the deep-learning systems outperformed the human grader (Chia et al., 2024). Additional prospective studies carried out by LOV have produced similar results with offline use of the AI diabetic retinopathy screening tool (Scheetz et al., 2021). Thus, implementing AI on the Sprinter van is evidence-based and provides an access opportunity for an underserved population with disproportionate rates of diabetic eye disease.

Another way the Sprinter van aims to catalyze advancements in bridging this gap is by harnessing the benefits of telehealth. Telehealth refers to the provision of real-time video consultations typically facilitated by a referrer. The consultation involves three parties: the patient, the referrer, and the specialist. LOV conducted retrospective and prospective audits of 5,456 patients to evaluate the cost of teleophthalmology. The results demonstrated that up to a quarter of patients seen in the Pilbara region were suitable for teleophthalmology, representing a potential saving cost of AUD 1.1 million per year (Razavi et al., 2017). With the launch of Starlink Roam in February 2023, a low-latency high-speed Internet service, the Sprinter van has been enabled to run a telehealth service and communicate with specialist ophthalmologists directly from any remote community in Western Australia. With previous population per ophthalmologist ratios in Pilbara being up to 19 times higher than the national average and the worst in Western Australia, teleophthalmology represents a tangible method to increase access of remote communities to an eye specialist (Australian Bureau of Statistics, 2021).

Navigating the Challenges of Historical Disenfranchisement in Healthcare

When considering how to maximize the impact of Lions Outback Vision in remote Western Australia, it is essential to acknowledge the historical disenfranchisement experienced by Aboriginal populations in the region. While in Australia for over 65,000 years, the last couple of centuries have seen these communities endure systemic inequalities, dispossession of their land, and removal from their families. This historical context is vital in understanding the legacy of mistrust toward charitable approaches from White Australians and represents a challenge for initiatives like LOV. The resilience of Aboriginal communities is a testament to their enduring spirit, but the scars of the past run deep. Recent political turmoil surrounding “The voice” referendum for Aboriginal population representation in parliament adds to this complicated history. In urban settings, Aboriginal rates of self-discharge are 5.7 times higher than their White counterparts because of the reluctance of the former to engage with Western medical practice (Askew et al., 2021). Poor medical literacy, exacerbated by little access to interpreters and limited cultural responsiveness of non-Indigenous clinicians, compounds this challenge faced by outreach services (Yashadhana et al., 2020). This context adds another layer of complexity to the acceptance and efficacy of LOV’s mobile clinics.

Complicating matters, LOV will have to overcome obstacles to produce solid evidence that the Sprinter van has a meaningful impact. Medical research and impact evaluation studies in Aboriginal communities are typically conducted from a Western knowledge perspective—the purported healthcare benefit, which is often elusive to Aboriginal people. To tackle this bias, the Aboriginal Health Committee of Western Australia (AHCWA), the leading authority for Aboriginal health in Western Australia, is committed to “promot[ing] and support[ing] quality research that will be reflective of the needs of the community,” placing culturally appropriate research with the priorities of Aboriginal people at its forefront (Aboriginal Health Council of Western Australia, 2024). Yet, the Committee has put in place a moratorium on impact evaluation studies of services such as LOV’s vision van, which from their perspective fail to fulfill the criteria of valuable research—that which strengthens capacities, increases opportunities for Aboriginal people, and improves health outcomes in accordance with Aboriginal values. By moving to the Broome hub along with two other ophthalmologists, Angus and his team hope to demonstrate a genuine commitment to tackling eye-care inequality while respecting Aboriginal values.

The Retina: A Window to the Entire Body

“The eye is truly becoming a window to the whole-body health,” says Angus. Backing this claim is leading-edge research using AI-enabled algorithms to detect features of other chronic diseases, ranging from cardiovascular to respiratory, in fundus photos. Information extracted from the tiny arteries and veins that supply the back of the eye can be analyzed and used to detect signs of cardiovascular disease and other life-threatening conditions (Rudnicka et al., 2022). Such development supports the idea that a Sprinter van embellished with AI and telehealth has excellent potential to establish a successful and innovative diabetic eye disease screening program that alleviates eye-care inequalities in a long-standing disenfranchised population. Granted future investment, there is scope for the Sprinter van to power multispecialty healthcare provision for Australia’s remote Indigenous patients.

The Final Countdown: How Will the Sprinter Van Make a Difference?

With 12 months to make a compelling case on the value of the Sprinter van, Angus has his job cut out for him. The vision van is effective, but only chips away at the disparities in eye health in remote Western Australia. Further, its effectiveness relies on the availability of local agents to remind Aboriginal people of their appointments and bring them to the van. Angus is now betting on complementing conventional van services with the smaller Sprinter. With this Sprinter van, LOV no longer needs to rely on complementary transport provisions for patients to reach remote local communities. However, since the Sprinter van only has space for two people, including the driver, there is a question about who should operate the AI-powered technology. Angus and his team must evaluate the advantages and disadvantages of allocating the second seat in the van to either a skilled medical professional, who is a scarce resource in Western Australia, or an Aboriginal Liaison Officer, who offers a vital cultural interface for effective community engagement. This decision will impact the operation of the fundus camera, the relay of results to the LOV Broome hub, and the overall success of the Sprinter van initiative in providing effective eye-care services.

More broadly, since the eye is becoming a window to body health—and eye health has a direct impact on the ability to advance the United Nations Sustainable Development Goals ([Figure 9](#))—the idea of equipping

mobile clinics with AI-powered eye-care technology and satellite Internet raises the question of whether the model offers a scalable solution to tackle inequality in healthcare for the whole of Western Australia and holds potential to address healthcare inequalities in remote regions of Africa, Brazil, India, China, and beyond. Angus cannot leave any stone unturned when it comes to demonstrating the potential of the Sprinter van. This might be the critical differential to win the Pilbara Challenge. Angus must act soon, or the LOV solution may end as fast as the blink of an eye.

Figure 9. The Link Between Improving Eye Health and Advancing the United Nations Sustainable Development Goals



Green arrows indicate relationships with direct evidence of a beneficial effect from improving eye health on SDGs. Dashed green arrows represent hypothesized direct beneficial effects. Black arrows represent possible indirect beneficial effects.

Source: The International Agency for the Prevention of Blindness. <https://www.iapb.org/blog/improving-eye-health-and-advancing-the-sdgs/>

Discussion Questions

1. What are the LOV's goals? Can you measure its performance? How?
2. What form of organization is LOV? What is the role of intersectoral collaboration in LOV?
3. What differentiates the vision van from the Sprinter van? What are the organizational implications of both approaches to tackle inequality in healthcare?
4. Who should go on the Sprinter van? Will it be better for an Aboriginal Liaison Officer to accompany the driver or a trained medic? What are the organizational impacts of differing choices?
5. Would the concept of LOV be as successful in other countries, institutional environments, and cultures? How much of LOV depends on the leadership of Angus Turner?
6. What role can you expect AI and other technological development to play in tackling grand challenges? How will such developments impact on organizational design choice?

Further Reading

For a recent overview on stakeholder management to tackle grand challenges, see:

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